



Name	Mrs.RAVINDER KOUR	Sample Received	28-12-2024 01:06 PM	Sample Collected At	LIFE SECURE CLINICAL LABORATORY
Age/Gender	75 Y / Female	Sample Processed	28-12-2024 01:07 PM	Referred By	Dr. Ametashver Singh, MBBS MD DM
UID	HLD2812202448223	Approved Date	28-12-2024 06:17 PM		

BIO CHEMISTRY REPORT

Investigations	Values	Unit	Bio-logical Reference
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RENAL FUNCTION SCREENING

*UREA	174.7	mg/dL	10.0 - 50.0
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Method: Enzymatic colorimetric

*CREATININE	8.1	mg/dL	0.4 - 1.1
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Method: Picrate Method

*URIC ACID	9.03	mg/dL	3.0 - 6.0
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Method: UV Method

*BLOOD UREA NITROGEN	81.64	mg/dL	5.0 - 23.0
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Method: Calculated

BUN/CREATININE RATIO	10.08	mg/dL	10.0 - 25.0
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Method: Calculated

UREA/CREATININE RATIO	21.57	mg/dL	20.0 - 50.0
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Method: Calculated

ELECTROLYTES SERUM.

*SODIUM	139	mEq/L	135.0 - 145.0
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Method: Ion Selective Electrode

*POTASSIUM	5.41	mEq/L	3.5 - 5.5
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Method: Ion Selective Electrode

*CHLORIDE	101.8	mEq/L	96.0 - 110.0
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Method: Ion Selective Electrode

Remarks: RESULT RECHECKED

The biological reference range pertains to the age and sex of the Patient.

Note: As the reports are computer generated, it must be treated as an original.

Dr. Anuja Sharma
 Consultant Pathologist
 Reg.No. 455

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BIO CHEMISTRY REPORT

***GFR, ESTIMATED**

RESULT: 4.4 mL/min/1.7m2

AGE IN YEARS	20 - 29	30 - 39	40 - 49	50 - 59	60 - 69	>=70
GFR IN mL/min/1.7m2	116	107	99	93	85	75

Normal GFR	Mild decrease in GFR	Moderate decrease in GFR	Sever decrease in GFR	Kidney Failure
>90	60 - 89	30 - 59	15 - 29	<15

Note:

1. National Kidney Disease Education program recommends the use of MDRD equation to estimate or predict GFR in adults (>=20 years) with Chronic Kidney Disease (CKD)

2. MDRD equation is most accurate for GFR <=60 mL/min/1.7m2

Critical value of Creatinine established as per laboratory policy: Adult : > 5.0

Such critical value if obtained needs urgent medical attention

Remarks: RESULT RECHECKED

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HAEMATOLOGY REPORT

Investigations	Values	Unit	Bio-logical Reference
*HEMOGLOBIN	8.66	g/dl	12.0 - 15.0

Method: Cyan-methemoglobin

Remarks:

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Consultant Pathologist
Reg.No. 455**A Unit Of Hexamed Healthcare Services LLP****Corporate Office:** Flat No 203, 2nd Floor, Sagacity Heights, Plot No 27, Kavuri Hills Rd, CBI Colony, Madhapur, Telangana 500033**Jammu Address:** 25-270/6, Main Bypass Road, NH-1A, Channi Rama, Jammu, J & K 180015

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CONDITIONS / PREREQUISITE OF LABORATORY REPORTING

It is presumed that the specimen belongs to the patient as identified in the test requisition form. The report will carry the name and age provided at the time of registration. The patient/referring doctor is requested to immediately notify any demographic error on the report with respect to Name, Gender, Age, Investigation report heading, etc., as may be necessary.

The reported results are best evaluated by the referring doctor or the concerned medical specialist.

Please note that results of the test(s) may vary from laboratory to laboratory depending on the technology and methodology used. Furthermore, some parameters may vary from time to time even for the same patient. Sometimes test(s) may have to be repeated in the interest of quality assurance. In some cases, the test(s) may be referred to another competent laboratory for cross verification of the results.

In certain instances, a second sample may be required from the patient on account of an intermediate result, or pre-analytical/analytical reason.

If your report carries a 'PRELIMINARY' status, it means that results for one or more test(s) are awaited specifically in case of microbiology tests. 'FINAL' culture report may be issued only after 2 to 3 days from the date of receipt of the sample, to facilitate its proper identification and the subsequent drug test(s) for its susceptibility.

Some tests are referred to other laboratories to provide a wider test menu to the patient. The details of the laboratory where the sample was referred to, can be obtained from the Customer Services desk of the facility. In such cases, the test reports may be delayed, which will be marked as # on the information booklet or information chart.

In the case of alarming and unexpected test results, you are advised to contact the laboratory immediately for further discussion and action. Laboratory results are meant to be correlated with the patient's clinical history. In such circumstances, please call 1800-212-6547.

Neither HexaMed Diagnostics & Specialty Clinics nor its directors/employees/ representatives assume any liability or responsibility, for any loss or damage or expenditure that may be incurred by any person, including the patient, as a result of assuming the meaning or contents of this report without further validation. Reporting of tests will be as per the defined laboratory turnaround time (TAT) for each test. The same will be informed to the patient during registration and / or sample collection. Delay in issuing test reports may occur due to unavoidable/unforeseen circumstances like shortage or non-availability of the test kits or instrument failure etc.

Please note: Histopathological specimens are retained for 6 months only from the date of the sample collection and blocks and slides are retained for 5 years from the date of its reporting. A minimum of 48 hours prior notice is required for the issuing the slides and blocks for such tests.

Test results are not valid for Medico Legal purposes. To maintain confidentiality, certain reports may not be sent over an email at the discretion of the management.

Reports can be received either as a hard copy or on an email on your personal mail id. Please note: Only reports of patients with no dues are sent over e-mail/uploaded or dispatched.

Home sample collection facilities are provided with prior appointment. Request for the same is to be made on 1800-212-6547 or booked online at www.hexamedhealthcare.com

In case of any queries pertaining to your test results or to provide feedback/suggestions please call us on 1800-212-6547 or email us at feedback@hexamedhealthcare.com

Claims if any, are subject to the jurisdiction of the Courts of Law in the Union Territory of Jammu and Kashmir.

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